

Psychosomatic (somatoform) disorders

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Charcot

Multiple symptoms





Overlap between Chronic Multi-symptom Illnesses

FIBROMYALGIA

2 - 4% of population;
widespread pain
and tenderness

MULTIPLE CHEMICAL

SENSITIVITY symptoms in
multiple organ systems in
response to multiple substances

CHRONIC FATIGUE

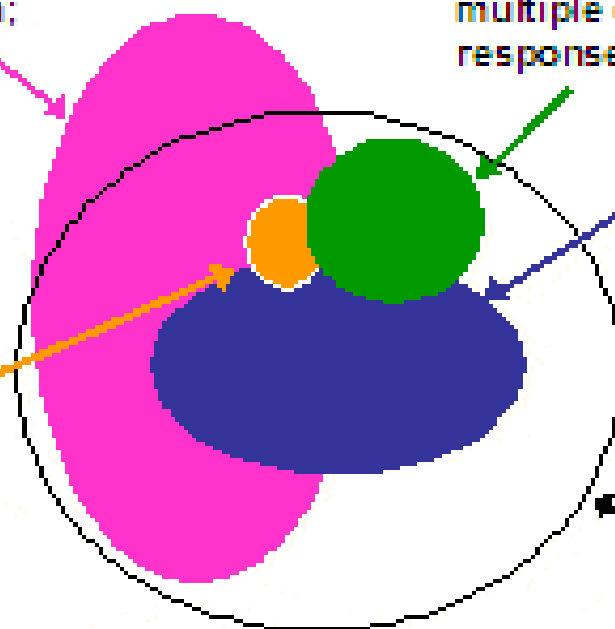
SYNDROME 1% of
population; fatigue and
4/8 "minor criteria"

EXPOSURE

SYNDROMES
Gulf War Illnesses,
Sick Building
Syndrome

SOMATIFORM

DISORDERS 4% of
population; multiple
unexplained
symptoms but no
organic findings



Somatic Symptom Disorder (DSM5 – 300.82)



- A. One or more somatic symptoms that are distressing or result in significant disruption of daily life
- B. Excessive thoughts, feelings, or behaviours related to the somatic symptom or associated health concerns as manifested by at least 1 of –
 - 1. Disproportionate & persistent thoughts about the seriousness of the symptoms
 - 2. Persistently high level of anxiety about health or symptoms
 - 3. Excessive time & energy devoted to these symptoms or health concerns

Somatic Symptom Disorder (DSM5 – 300.82)



C. Although any one somatic symptom may not be continuously present, the state of being symptomatic is persistent (typically > 6/12)

Specifiers:

- i. **With predominant pain** (the old Pain Disorder)
- ii. **Persistent** (severe symptoms, marked impairment and long (>6/12) duration)
- iii. **Mild** (1 from B); **Moderate** (2 or more from B); **Severe** (Moderate + multiple somatic complaints or one very severe)



Somatization (somatoform disorder): ICD-10

- Multiple/variable physical symptoms without adequate physical basis for 2 years or more [<2 years, 'undifferentiated']
- refusal to accept explanation or reassurance of several doctors
- impaired social/family functions due to symptoms or behaviour



Somatization (somatoform disorder): DSM-IV

- A: similar to ICD-10; onset <30 yr
- B: 4 pain symptoms (4 sites or functions) + 2 GI symptoms + 1 sexual symptom + 1 pseudo-neurological symptom
- C: No physical explanation OR symptoms>>>signs
- D: Not intentionally produced

Somatization: epidemiology



- 20% of GP patients with new illness fulfill Somatization criteria
- 70% have physical condition which can not account fully for symptoms
- <20% have psychiatric disorder
- 60-80% fear serious illness (cf. ~20% of those with same symptoms who do not consult GP)

Somatization: psychology



- Belief that symptoms = illness
- Use of ‘reassurance’
- Reinforcement by family or friends
- Presence of anxiety, depression or other physical symptoms may cause/exacerbate symptoms
- Psychological precipitant (stress, life events, freedom from symptoms: weekends & holidays)

Somatization: treatment principle



- Aim is to ***re-attribute*** symptoms from physical to psychological cause

Pain



- Pain is subjective
- Pain is difficult to measure
- Pain is difficult to refute
- Pain is “a poor solution” to internal conflicts (Merskey)
- Pain is difficult to link with psychological mechanisms

Pain behaviour



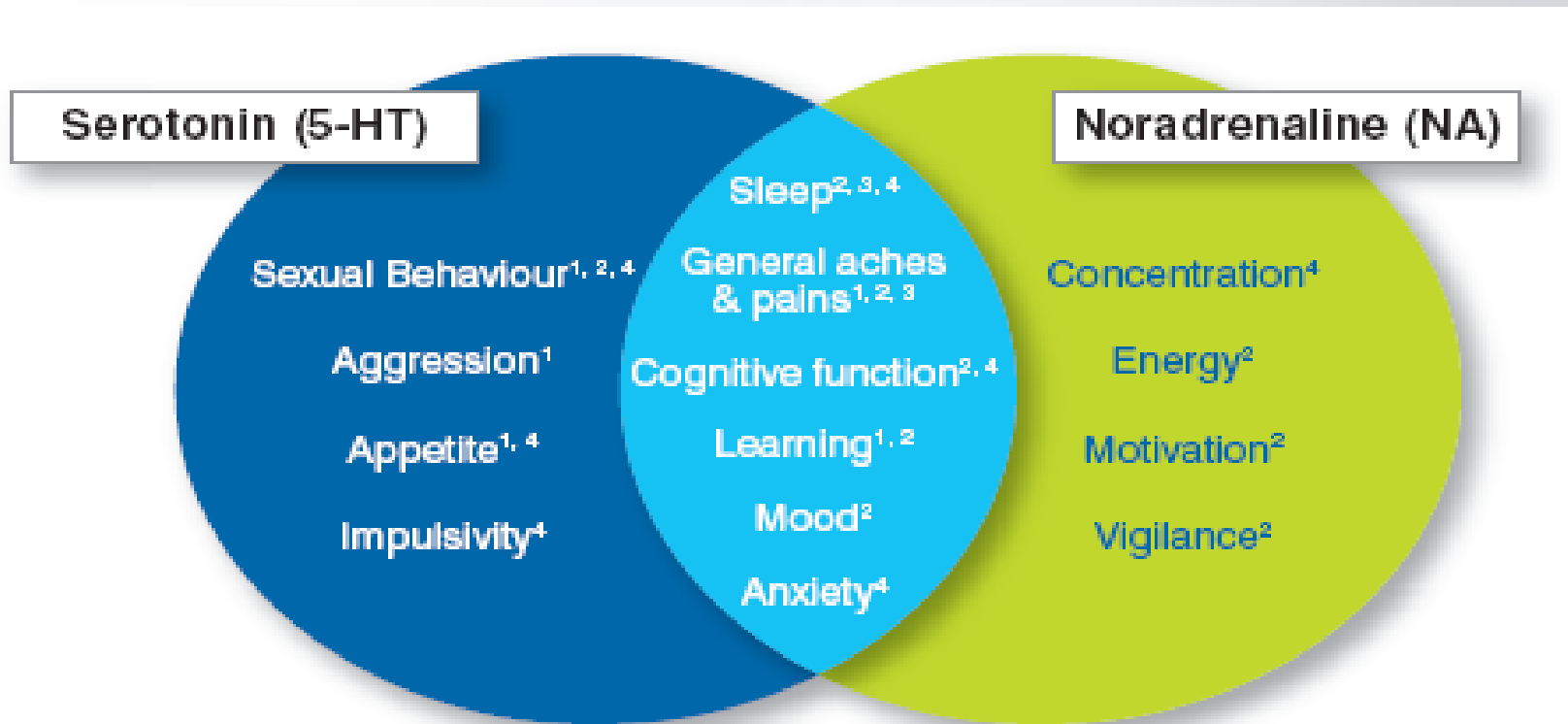
- Dependency; passivity
- 'Catastrophizing'; attentional diversion
- Guarding; avoidance of activity
- Nonverbal expressiveness
- Belief in uncontrollable nature of pain
- Anticipatory use of analgesics

Gender and pain



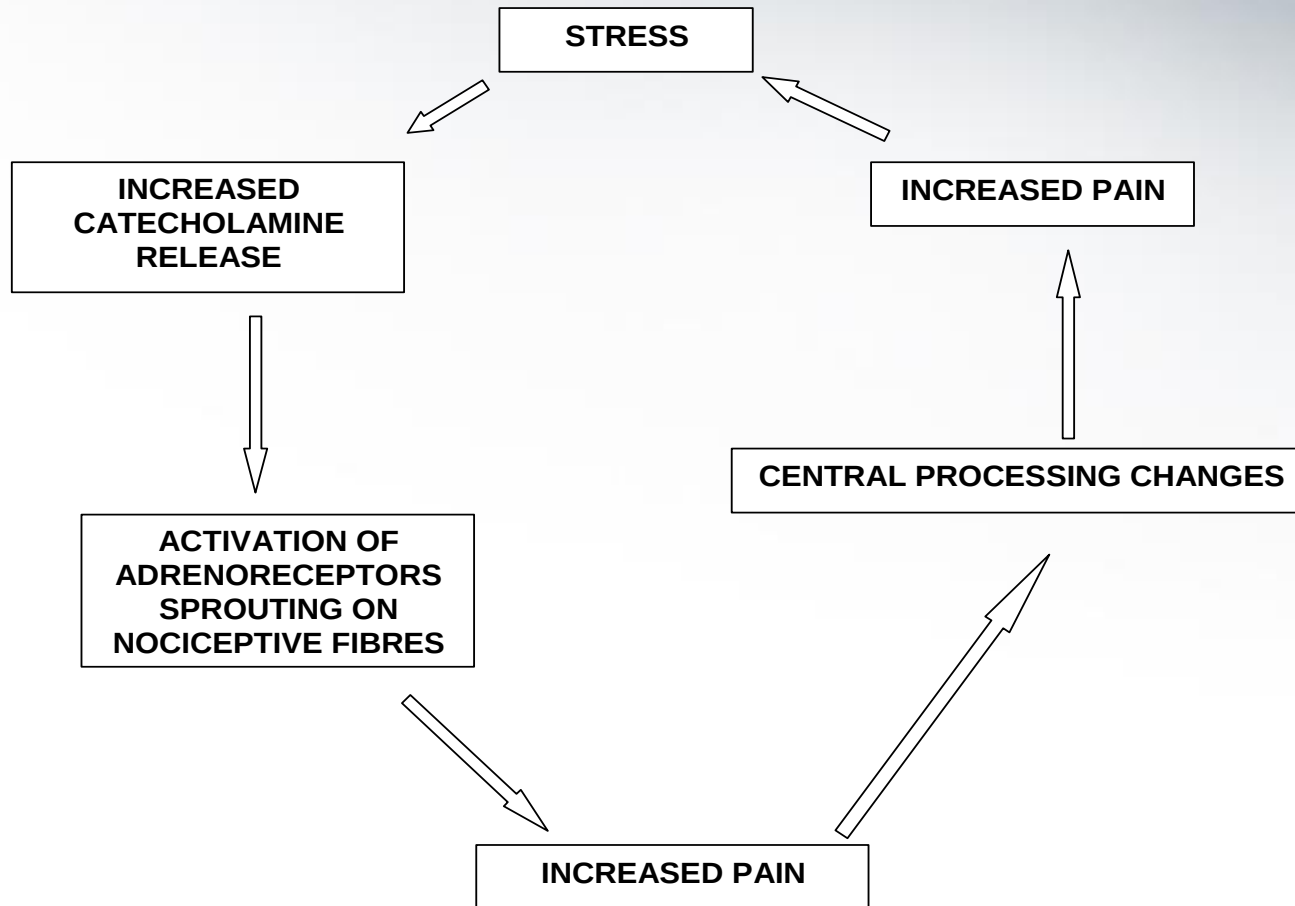
- Sex differences in most (all?) sensory systems
- Higher prevalence in females of headache, fibromyalgia, TMD, IBS
- Influence of reproductive status?
- Mu-opioids > in males (rodents)
- Kappa-opioids > in females (rodents)

5-HT & NA* in pain



1. Lucki I. *Biol Psychiatry* 1998; **44**: 151–162.
2. Frazer A. *J Clin Psychiatry* 2001; **62**(Suppl 12): 16–23.
3. Jones CL. *Prog Brain Res* 1991; **88**: 381–394.
4. Ressler KJ, Nemeroff CB. *Depress Anxiety* 2000; **12**(Suppl 1): 2–19.

Vicious cycle of pain & arousal



adapted from Breuhl 2001

Pain & depression



- Prevalence of depression in chronic pain 11-100%
- 66% of patients with medically-unexplained pain
- Affective distress v. affective disorder
- Greater pain intensity, reduced quality of life, less life-control, early retirement, pain behaviour
- Depression is a good predictor of disability
- Depression affects closure of litigation
- Increased risk of suicide & self harm

Depression & pain



		Participants affected (%)
Psychological	Symptom	
	Depressed / sad / low mood ¹	76
	Emotional / cry / want to cry ¹	59
	Anxious / nervous / fearful ¹	57
	Poor memory / poor concentration ¹	51
	Irritable ¹	50
Somatic	Feel / felt worthless ¹	40
	Tired / no energy / listless ¹	73
	Concomitant medical condition (e.g. backache, arthritis) ¹	65
	Broken sleep / decreased sleep ¹	63
	General aches and pains ²	43
	Change in appetite ¹	40
	Palpitations / feeling heart is going too fast ¹	34

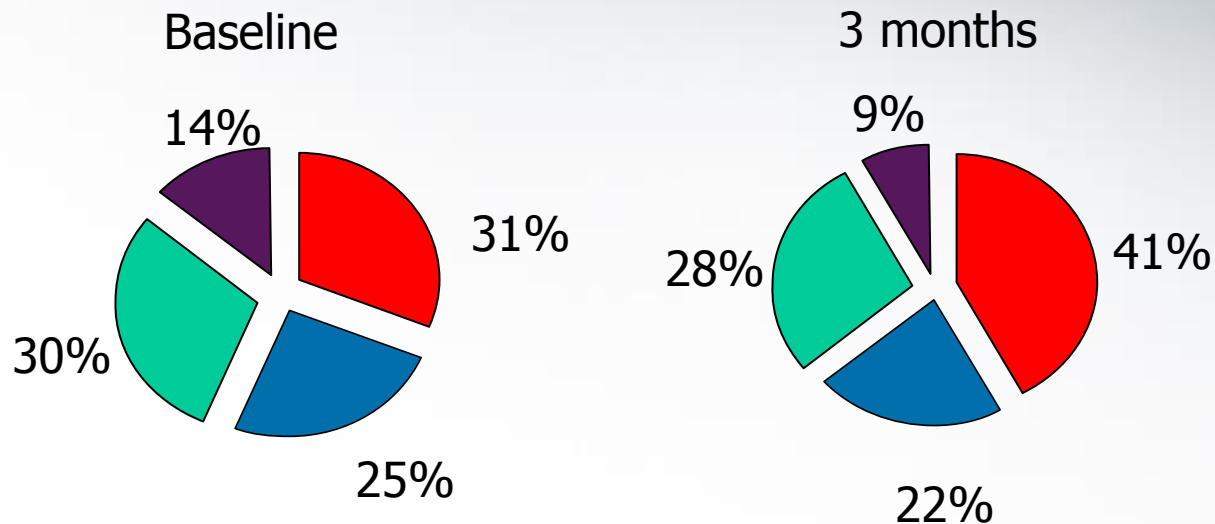
1. Tylee A *et al.* *Int Clin Psychopharmacol* 1999;**14**: 139–151.

2. Ohayon MM, Schatzberg AF. *Arch Gen Psychiatry* 2003;**60**: 39–47.

Impact



Pain was very commonly experienced in this sample of depressed patients and improved modestly with treatment for 3 months



■ None ■ Mild ■ Moderate ■ Severe

All patients were treated for 3 months with fluoxetine, paroxetine or sertraline

N = 573

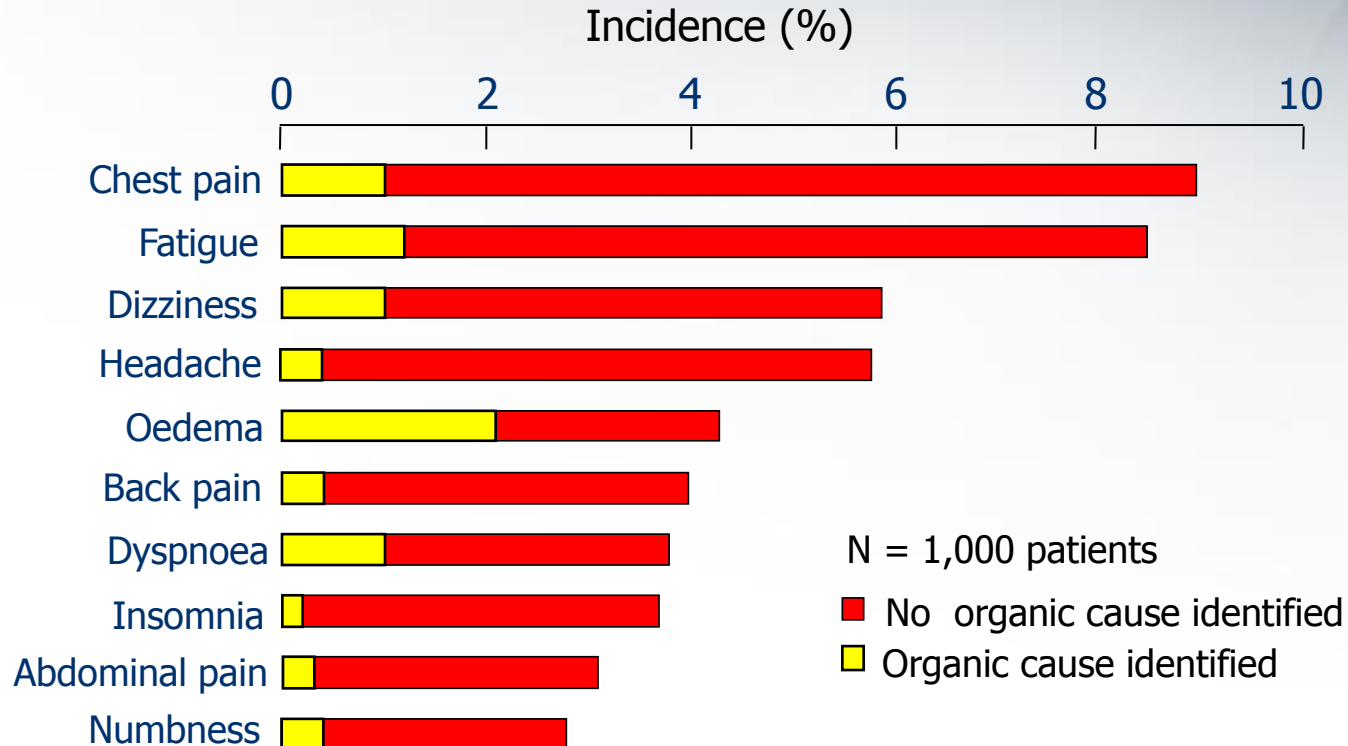
Baseline SCL-20 = 1.66

Improvement SCL-20 to 0.9 at 3 months

Origin of symptoms



Symptoms in ambulatory care patients over a 3-year period

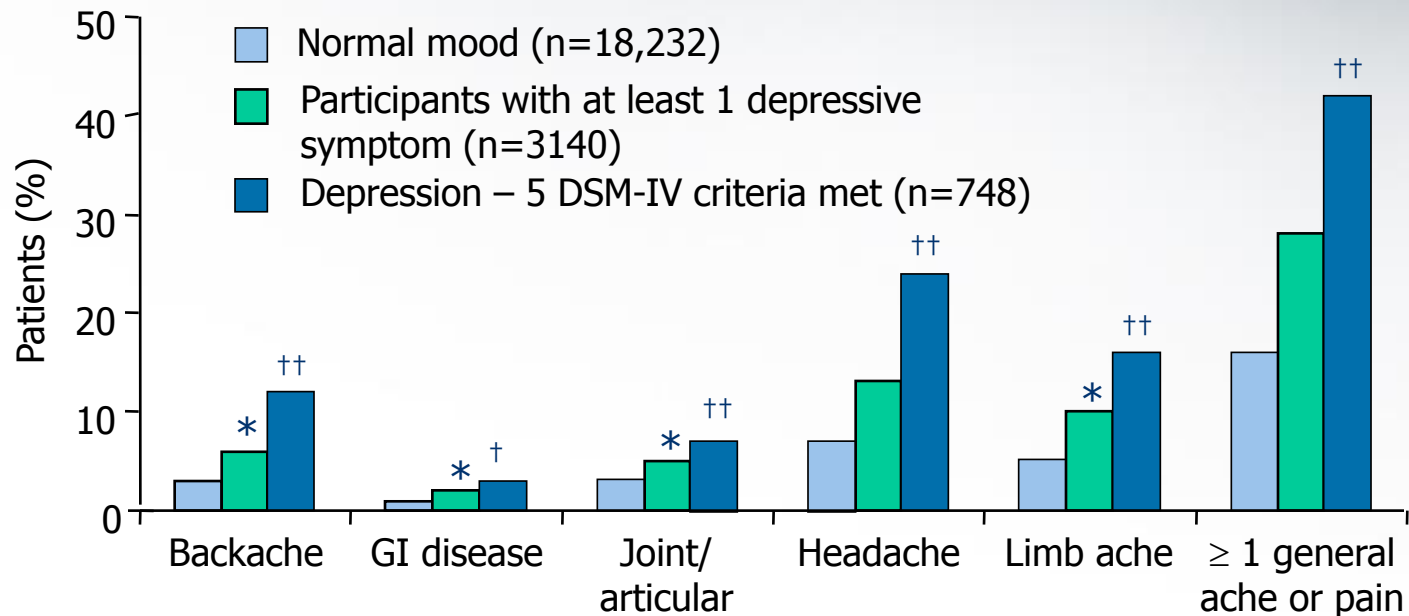


Statistical analyses were conducted using SPSS-X, 1983

Frequency of somatic symptoms



Depressed patients are four times more likely to experience general aches and pains than people without depression¹

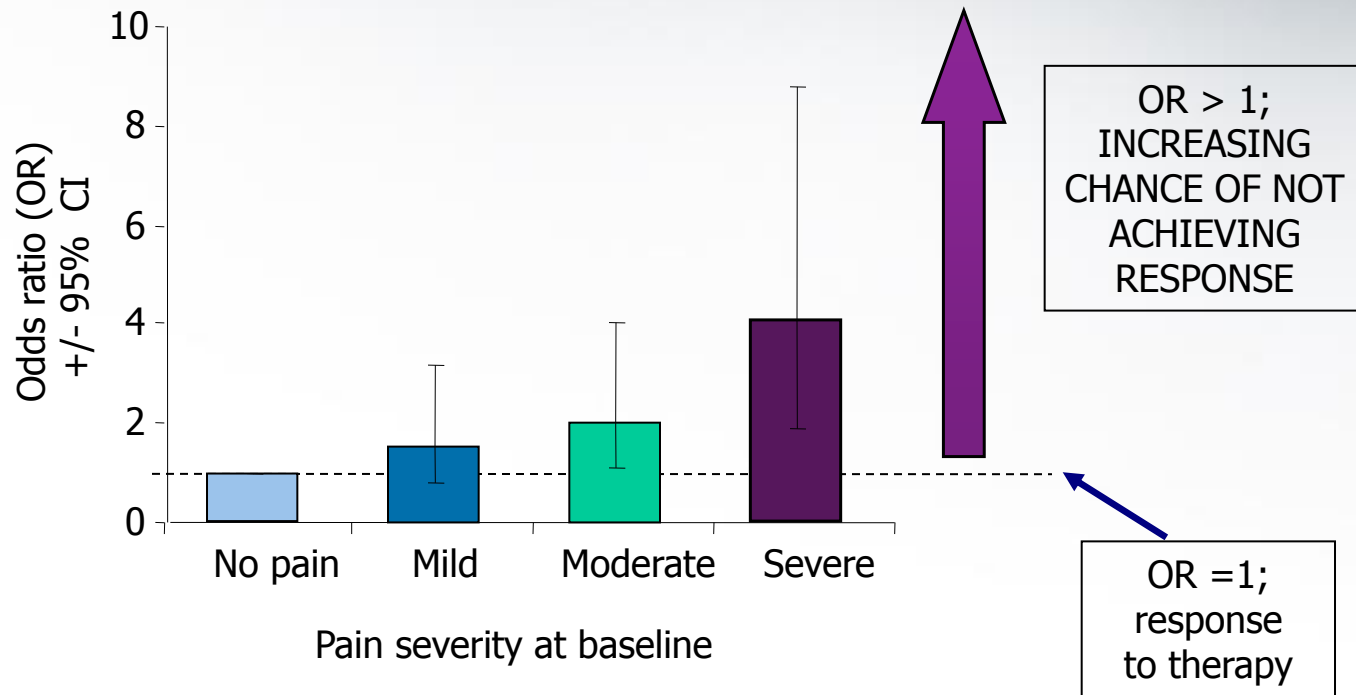


* $p < 0.001$ vs. patients with normal mood

^{††} $p < 0.001$, [†] $p = 0.03$ vs. patients without depression

Logistic regressions were performed using software which estimates the SEs from stratified samples using a Taylor series linearization method

Impact & response

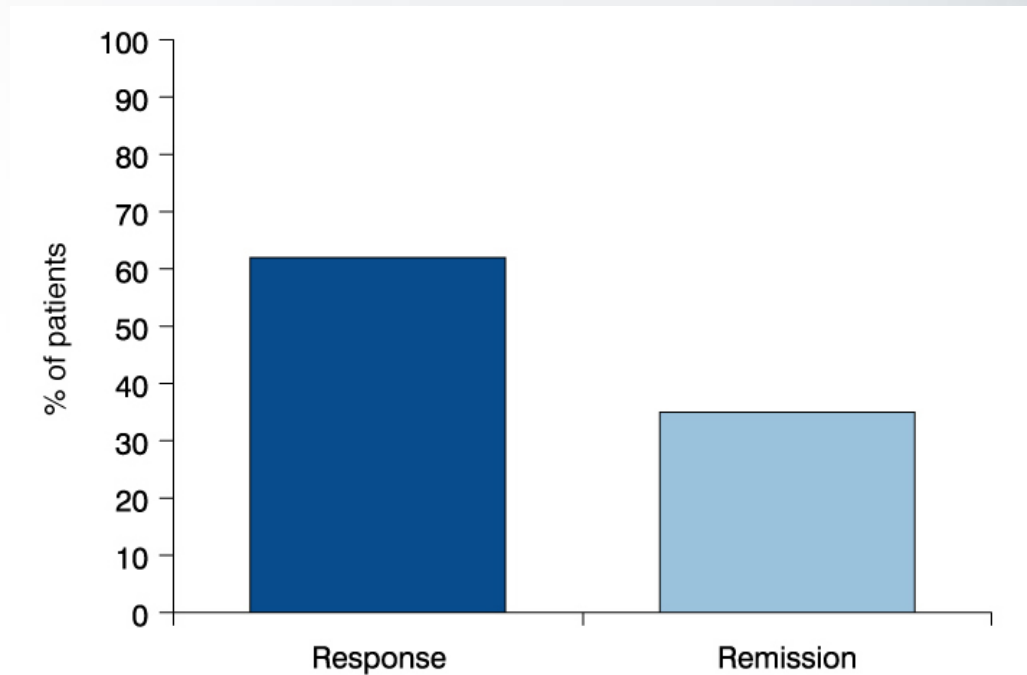


N=573
SF-36 scale

Current treatment outcomes



Approximately 2/3 of patients treated for depression will fail to achieve remission ($\text{HAM-D} \leq 7$)²



1. Fawcett J, Barkin RL. *J Clin Psychiatry*. 1997; **58**(Suppl 6): 32–39.
2. O'Reardon JP, Amsterdam JD. *Psychiatr Ann* 1998; **28**: 633–640.

Reasons for 'poor' outcome



- Under-recognition¹
- Under-treatment^{1,2}
- Poor treatment adherence¹
 - A total of 50% of patients receiving an initial prescription for an antidepressant stop treatment in the first month¹
 - Average drop-out rates are lower in trials of newer antidepressants compared with older agents (primarily tricyclic antidepressants)³
- Poor patient education⁴
- Lack of regular follow-up¹

1. Cassano P, Fava M. *J Psychosom Res* 2002; **53**:849–857.

2. Hirschfeld RMA, Keller MB, Panico S, *et al.* *JAMA* 1997; **277**(4):333–340.

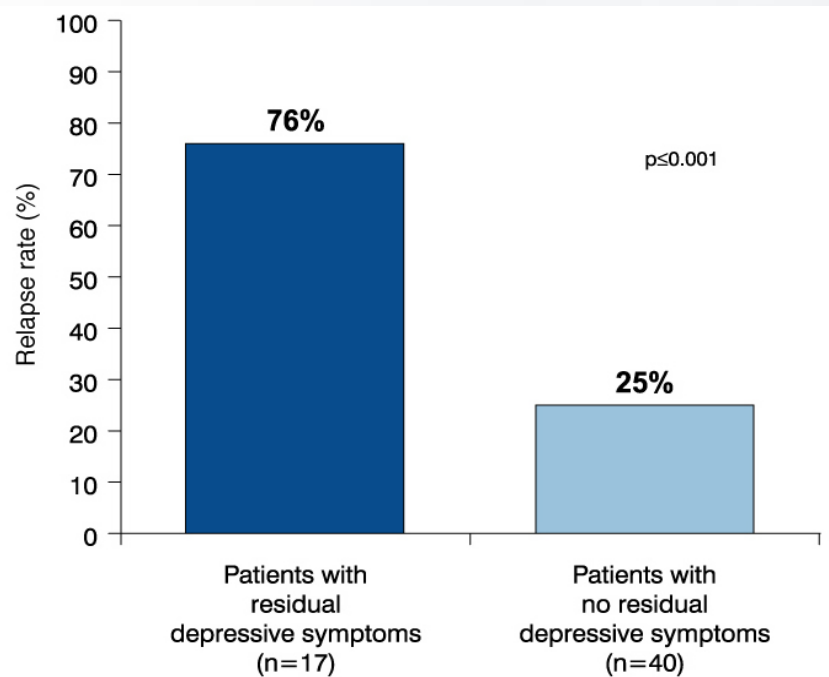
3. Mulrow CD, Williams JW Jr, Chiquette E, *et al.* *Am J Med* 2000; **108**(1):54–64.

4. Zajecka JM. *J Clin Psychiatry* 2003; **64**(Suppl 15):7–12.

Consequences of non-remission



Depressed patients with unresolved symptoms
are 3 times more likely to relapse¹



Patients were interviewed
at 3-monthly intervals
according to the Clinical
Interview for Depression
and HAM-D₁₇ Rating Scale

Adapted from Paykel ES, et al. 1995

Quality of life



- In patients with depression, lower perceived physical and social functioning QoL ratings at baseline were associated with lower treatment response
- SF-36 scores were significantly higher* in remitters than in partial responders for:
 - Physical functioning (79.0 ± 13.7 vs. 69.9 ± 13.7 , p 0.004)
 - Social functioning (79.9 ± 19.5 vs. 70.6 ± 29.5 , p 0.09)

Remission associated with normalisation of QoL domains

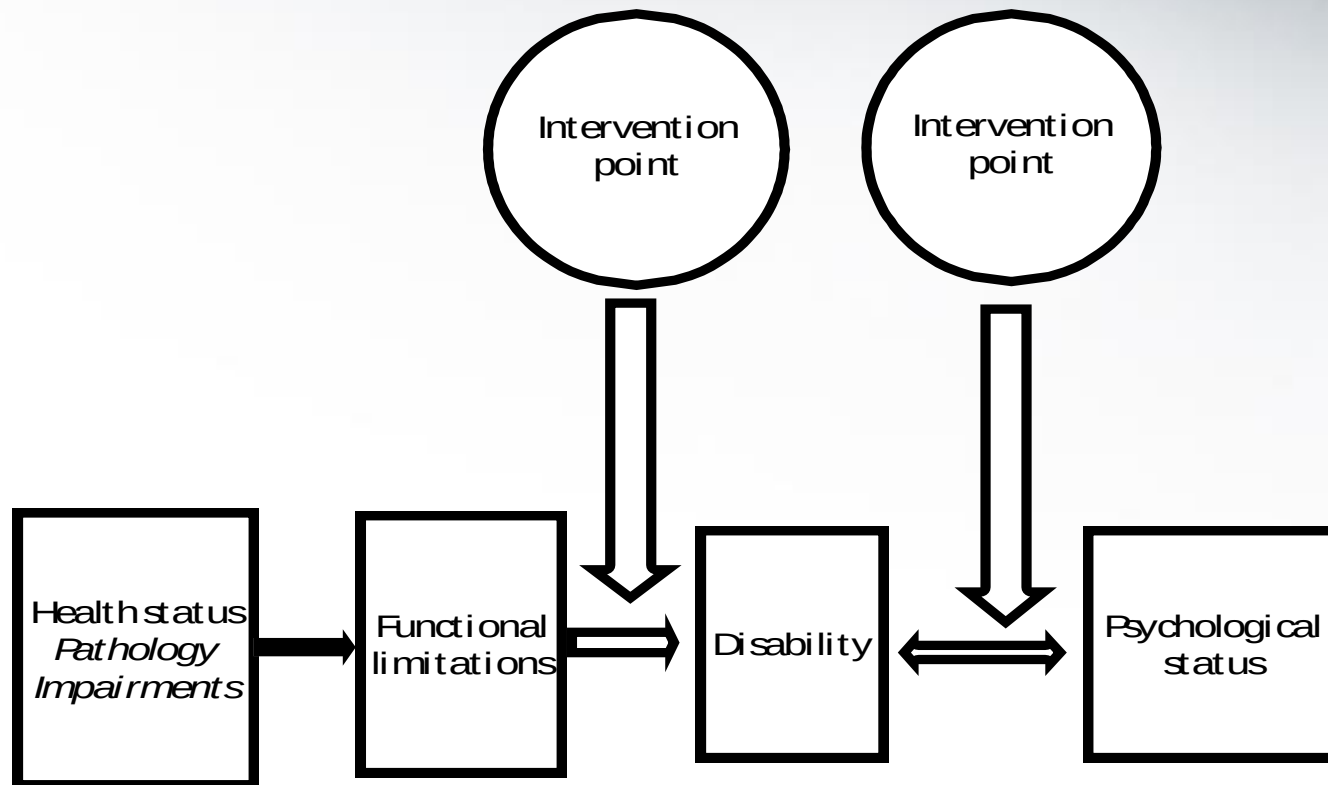
*SF-30 higher score = better QoL

Other psychiatric disability



- Anxiety (50% symptoms; 30% anxiety disorder)
- Migraine: increased risk (OR 12.8) panic disorder
- Fear-avoidance and expectancy
 - Fear of pain, movement, re-injury
 - Reinforced by low self-esteem, catastrophising
 - Significant predictor of failure to return to work
- High levels of catastrophic thinking & fear-avoidance predict prolonged disability
- Anger

Intervention & disability



Deconstructed, parsed, and diagnosed.



Deconstructed, parsed, and diagnosed.

A hypothetical example illustrates how precision medicine might deconstruct traditional symptom-based categories. Patients with a range of mood disorders are studied across several analytical platforms to parse current heterogeneous syndromes into homogeneous clusters.

Symptom-based categories

Major depressive disorder



Mild depression (dysthymia)



Bipolar depression



Integrated data

Genetic risk
polygenic risk score

Brain activity
insula cortex

Physiology
inflammatory markers

Behavioral process
affective bias

Life experience
social, cultural, and
environmental factors

Data-driven categories

Cluster 1



Cluster 2



Cluster 3



Cluster 4



Prospective
replication and
stratified clinical
trials

Thomas R. Insel, and Bruce N. Cuthbert *Science*
2015;348:499-500



Psychiatric causation



- Not the same as legal causation!
 - *Verstehen* (empathetic understanding) and *Erklären* (causal) reasoning – Jaspers
 - *Verstehen* is based on the meaning of events as we intuitively understand them
 - *Erklären* is (supposedly) based on scientific reasoning
- “Interpretations ... taken as explanations, understanding ... conflated with causation ... what may be meaningful is taken as causal” - Trimble

Causation



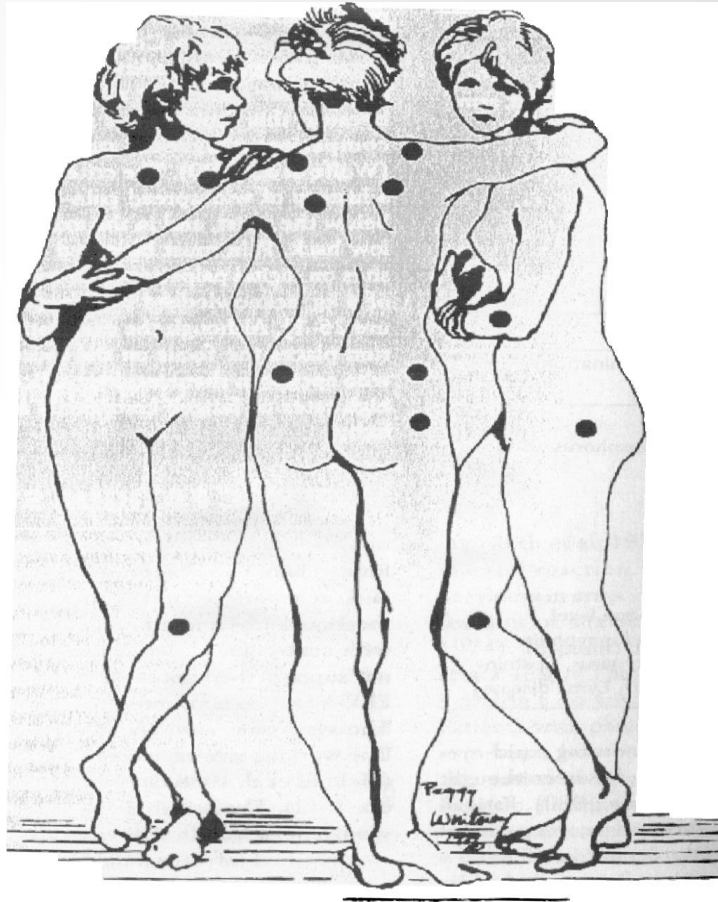
“In the days of the Malleus, if the physician could find no evidence of natural illness, he was expected to find evidence of witchcraft: today, if he cannot diagnose organic illness, he is expected to diagnose mental illness.” - Szasz

Fibromyalgia



- Generalised pain
- Specific tender points (11/18); force of 4kg; acknowledged by patient to be painful
- Family history of mood disorder?
- Responsive to antidepressants?
- Responsive to CBT?

Fibromyalgia: trigger points



Wolfe et al 1990 *Arthritis & Rheumatism* 33: 160-172

Fibromyalgia



- Generalised pain
- Fatigue and sleep disturbance
- Irritable bowel syndrome
- Headaches & facial pain (may be migraine-like)
- Heightened sensitivity
- Other symptoms
 - Anxiety, depression & mood swings
 - Poor concentration
 - Painful menstruation, dry eyes, dizziness, chest pain

Fibromyalgia: risk factors



- Women >> men
- Early & middle adulthood
- Sleep disorder (including sleep apnoea) [?]
- Family history of chronic pain syndromes, especially fibromyalgia
- Rheumatic disease

Fibromyalgia: treatment



- Medication (especially antidepressants)
- Cognitive-behavioural therapy (CBT)
- Residential treatment programmes
- Psychotherapy



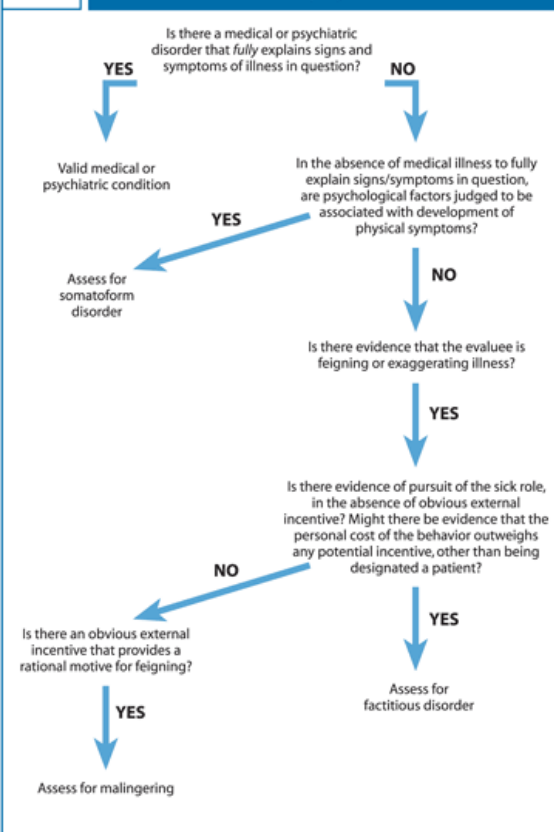
Faking it?





Figure

Guide to help distinguish malingering, factitious disorders, somatoform disorders, and valid medical/psychiatric conditions



Factitious disorder



- Conscious 'forgery' of illness
- 'Hospital addiction'
- 'Internal' secondary gain, through emotional satisfaction, etc.
- 'Specialisation'
- Pseudologia fantastica
- Peregrination

Malingering



- Conscious deceit with external gain (compensation, benefits, drugs, etc.)
- Continuum with conversion disorder?
- No guaranteed objective test or instrument for detection
- Personality assessment helpful
- Contradictions rather than discrepancies

'Tests' of malingering



- TOMM
- Effort Testing
- SIRS
- Atypical Response Scale – Trauma Symptom Inventory
- MMPI-2 subscale
- MENT
- &c

Lying



- Mouth cover
- Nose touch
- Eye rub
- Ear grab
- Neck scratch
- Collar pull
- Fingers in the mouth

The good liar



- Well prepared
- Original
- Quick-thinking
- Eloquent
- Good memory
- No fear, guilt or delight
- Good actor

Lie detecting



- Non verbal communication
- Verbal communication (speech analysis)
- Physiological measurement
- Neuroimaging (fMRI)

Lie detecting: non verbal



- Liar experiences cognitive dissonance
- Lie expressed in
 - Stutters
 - Pauses
 - Slower rate
 - Higher pitch
 - Rigidity
 - Facial expression

Lie detecting: non verbal



- Individual differences
- Equipment needed?
- Lay people detect poorly, men < women
- Experts spot 60% truth; 92+% lies
- Difficult to fool observers

Lie detecting: verbal



- Liar experiences cognitive dissonance
- Lie expressed by
 - Implausible answers
 - Negative statements
 - Indirect answers
 - Lack of personal experiences
 - Structured production
 - Lack of detail

Lie detecting: verbal



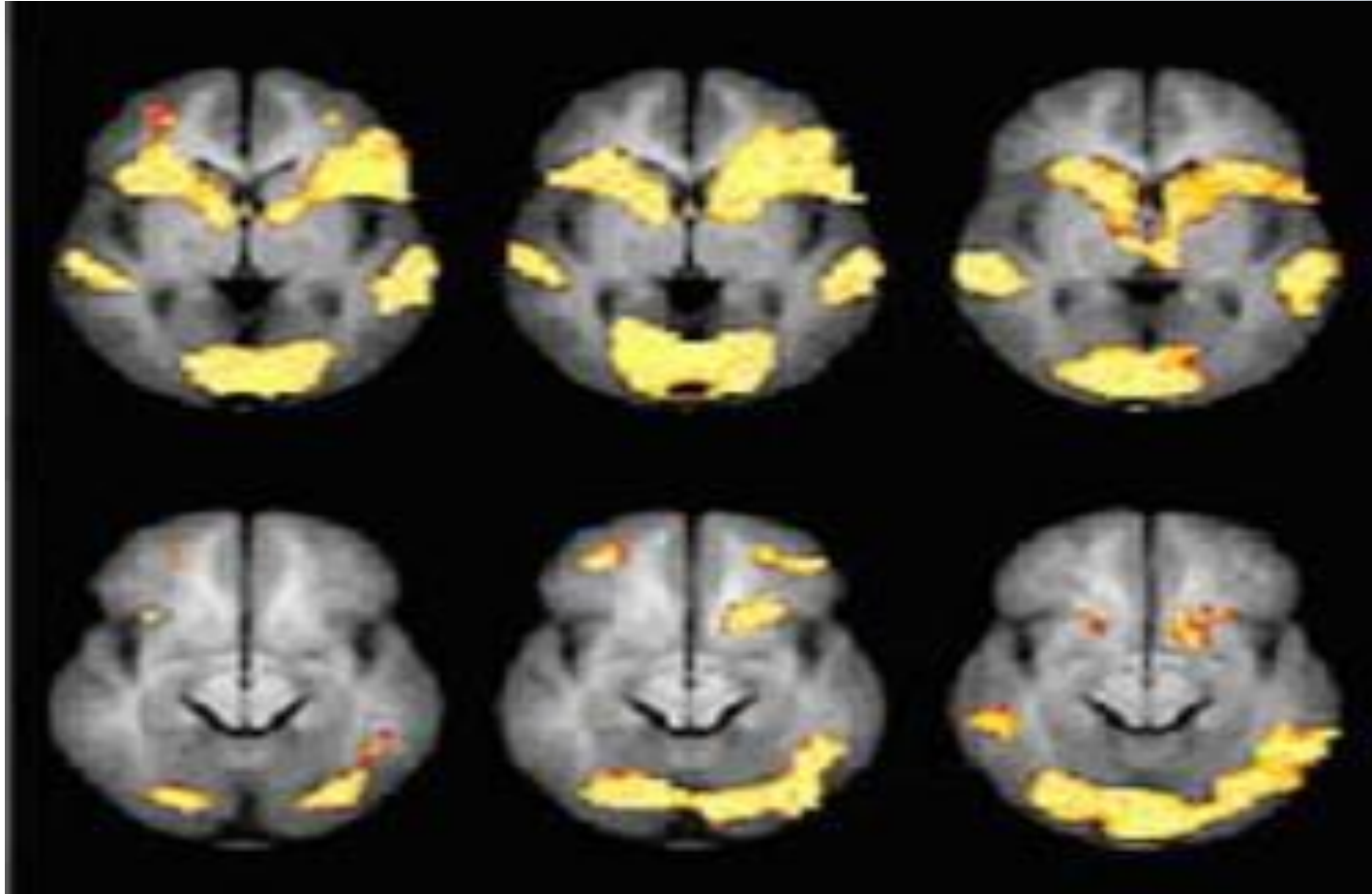
- Individual differences
- No equipment needed
- We don't know how good lay people are at detecting verbal lies
- Experts detect better than chance
- Not all that difficult to fool observers

Lie detecting: physiological



- Physiological tests
- Quite easy to fool
- Better than chance detection in some studies

Lying: fMRI



Thank you!

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